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Correct Answer

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Practice Exam - 1

Question 114 of 117



A 65-year-old male with a history of paroxysmal atrial fibrillation, type 2 diabetes mellitus, hyperlipidemia, ischemic cardiomyopathy with an LVEF of 30% s/p ICD, presents to the emergency room with lower extremity edema, dyspnea on exertion and paroxysmal nocturnal dyspnea. He is warm to touch on exam, jugular venous distention is at 18 cm above the clavicle, + hepatojugular reflux. HR 97 bpm, BP 117/53 (74) mmHg, RR 18 breaths/min, Saturation 97% on room air. His NT-proBNP > 5000, creatinine is 1.2 (baseline is 0.8). He reports not having taken his diuretic over the past three days.

His home medications include: furosemide 40 mg daily, metoprolol succinate 25 mg daily, spironolactone 25 mg daily, sacubitril/ valsartan 49/51 mg Bid, dapagliflozin 10 mg daily.

You decide to diurese him with furosemide 80 mg IV. The next day, you notice the serum creatinine increases to 1.4 mg/dL, and he is net negative 500 mL only., his exam is unchanged. You administer 120 mg IV Bid. The following morning serum creatinine is now 1.6, and the exam is unchanged.

What is the best course of action?

A. Continue all home medications



B. Initiate Nitroprusside



C. Order a right heart catheterization

D. Administer a fluid bolus

E. Initiate Milrinone

Commentary

References

Teaching Point: AHFTC should know indications and appropriate timing for right heart catheterization referral.

Correct Answer: C. Order a right heart catheterization

Rationale: The serum creatinine has doubled, and more importantly increased with the administration of diuretics. Persistent congestion despite escalating diuretics, worsening renal function, and poor diuretic response warrant right heart catheterization to clarify hemodynamics and guide further therapy.

Incorrect answers:

- A. Given worsening kidney function, home medications and particularly spironolactone, sacubitril/ valsartan and dapagliflozin should be held.
- B. The data in the case does not provide enough information to start the use of nitroprusside.
- D. The patient is already fluid overloaded.
- E. Milrinone should not be routinely started without the use of right heart catheterization and further understanding of hemodynamic profile.

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